

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

A data-driven overview for public health policy analysts and
mental health program planners.

Source: Our World in Data — Mental Health
Authors: Dattani, Rodés-Guirao, Ritchie, and Roser (2023)
Data source: IHME, Global Burden of Disease (2025)



DALYs per 100K
across five categories

Hundreds of Millions Affected: Major Depression Risk Is High

1 in 3

women will experience major depression in their lifetime



1 in 5

men will experience major depression in their lifetime



Mental health conditions span a spectrum

Common disorders such as depression and anxiety affect very large populations.

Less common conditions such as schizophrenia and bipolar disorder can have large impact on individuals' lives.

Policy relevance: plan for scale and severity at the same time.

Depression and Anxiety Dominate DALYs per 100,000 People

World, 2023 • disability-adjusted life years combine years of life lost and years lived with disability.

Depressive disorders



Central pattern

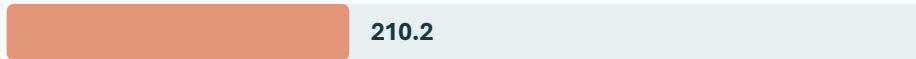
6

Anxiety disorders



~80%

Schizophrenia



210.2

Bipolar disorder



94.6

Eating disorders

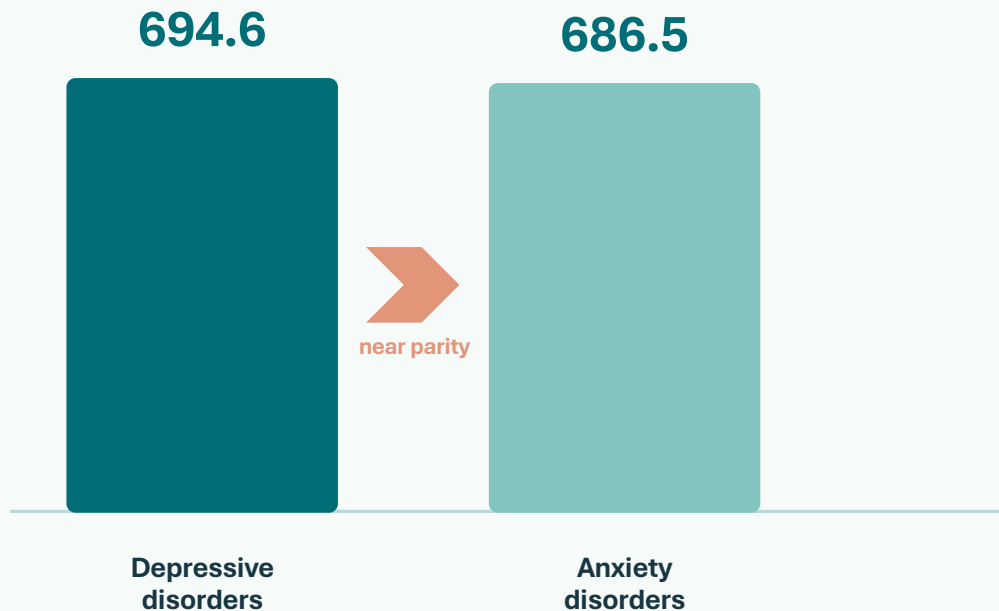


47.5

DALYs per 100,000 people

of total burden
across these five
categories comes
from depression and
anxiety combined

Depression and Anxiety Carry Nearly Equal Disease Burden

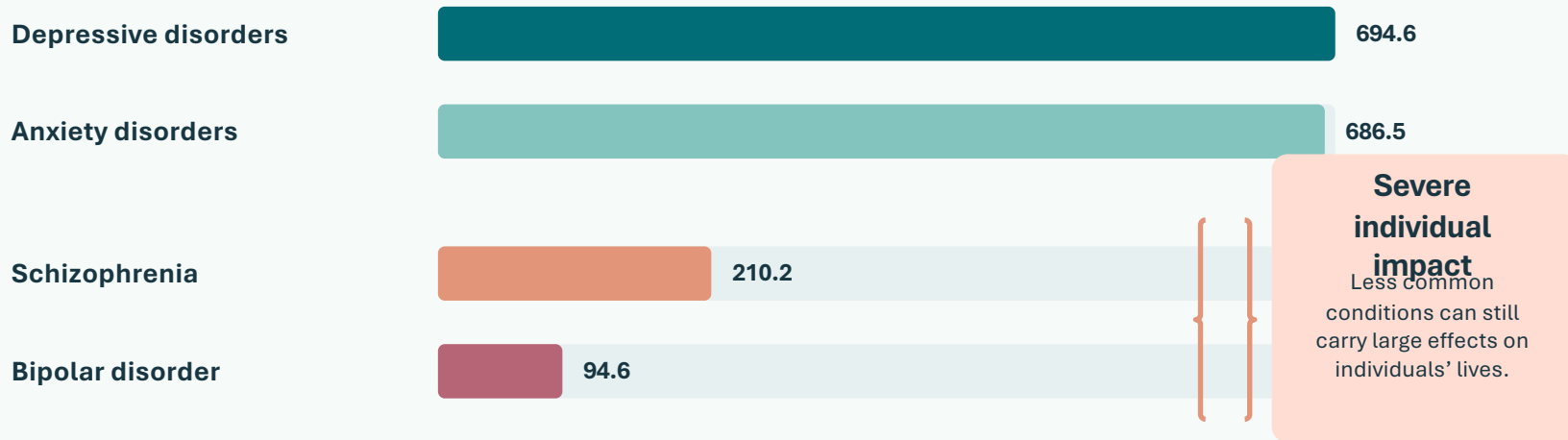


Implication for planning

Depressive and anxiety disorders each generate approximately 690 DALYs per 100,000 people.

Together, they dwarf the other mental illness categories in the data table.

Schizophrenia and Bipolar Disorder: Lower Population Burden, Severe Impact



Schizophrenia contributes 210.2 DALYs per 100,000, reflecting severe disability associated with the condition.



Bipolar disorder contributes 94.6 DALYs per 100,000 and remains part of specialized-care planning.

Treatment Is Available but Often Lacking or Poor Quality

Mental illnesses are treatable and their impact can be reduced — but barriers persist.

S

Stigma

Many people are uncomfortable disclosing symptoms to healthcare professionals or people they know.

A

Access

Treatment is often lacking, especially in low- and middle-income countries.

Q

Quality

Even where treatment exists, it may be of poor quality.

U

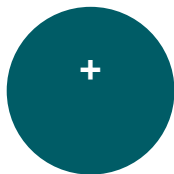
Underreporting

Discomfort with disclosure makes true prevalence difficult to estimate.

The actual burden may be higher than reported figures suggest.

How People Cope: Medication, Faith, and Family Conversations

Global survey data show wide variation in how people deal with anxiety or depression.



Taking prescribed medication

Common in higher-income settings



Religious or spiritual activities

Prevalent in many regions worldwide



Talking to friends or family

A widely reported coping mechanism

Policy interventions must account for cultural, economic, and healthcare system differences rather than applying a one-size-fits-all approach.

Key Takeaways for Policy and Planning

01

Depression and anxiety represent the overwhelming majority of DALYs across the five mental health categories.

02

Schizophrenia and bipolar disorder carry disproportionate individual burden and require continued specialized care.

03

Treatment exists, but access and quality remain inadequate in many settings.

04

Stigma suppresses help-seeking and makes the true prevalence difficult to estimate.

05

Scale interventions for common mental disorders while maintaining care for severe conditions.

Plan for scale + severity